

Patient's Name: _____
UHID: _____ IP ID: _____ Age: _____
Gender: Male/Female _____ D.O.A.: _____ Unit: _____

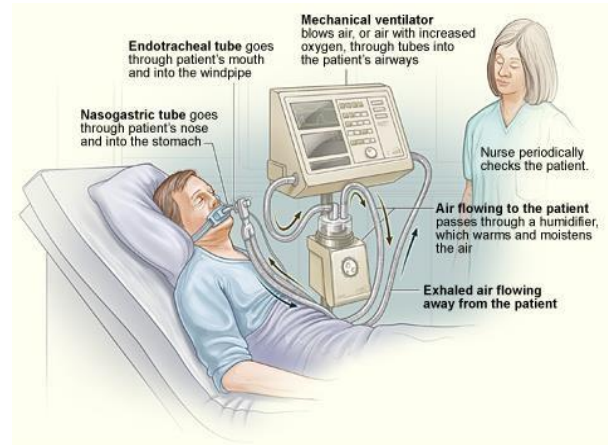
Informed Consent for Intubation and Mechanical Ventilation.

I, _____ ☐ the Patient, or ☐ (representative of patient
_____ (please tick the correct option

above) have read and been explained in a language which I fully understand and have understood that the patient is diagnosed as _____ and requires

Intubation and Mechanical Ventilation.

Intubation means a tube is put into the windpipe (trachea) to protect the airway and help one breathe. Insertion of endotracheal tubes is done using a laryngoscope, an instrument used to view the upper portion of the trachea & below the vocal cords. The tube may be attached to a machine called a ventilator or respirator that breathes for the patient by moving air into and out of his/her lungs. This is called mechanical ventilation.



One may need intubation and mechanical ventilation if one is not able to breathe on his/her own or get enough air into his/her lungs when one breathes. The ventilator delivers oxygen into the lungs a set number of times per minute. The heart rate and oxygen level are monitored during the procedure.

Intended benefits:

Helps unconscious patients to protect their airway/assist controlled breathing through Ventilator, Management of respiratory failure, Airway protection, Protects lungs, Prevents obstruction of the airway & Suffocation

Possible risks and complications:

Trauma to the vocal cord, trachea, voice box, and thyroid gland, Pneumothorax (Air Around Lungs) , Lung Collapse Bleeding, Infection, Perforation of Trachea.

Possible alternatives:

Bipap Ventilation, Laryngeal mask Airway , Oxygen by mask, Tracheostomy Esophageal Tracheal Combi tube

Post procedure course:

The blood oxygen level is monitored by a sensor that is attached to the finger or earlobe & regular blood tests. The treating doctor may prescribe medicine to Treat pain / sedate the patient to keep him/her from moving around too much and pulling the tube out / Help relax his /her airways / reduce swelling in the airways. The recovery is monitored to see if the patient can begin to breathe on his/her own. The ventilator settings are gradually changed to allow the patient to do most of the work of breathing, with support from the ventilator as needed & the breathing tube will be removed if patient is breathing well enough on his/her own & there is no longer a need to protect the airway. I have understood the information provided about **Intubation & Mechanical Ventilation** in this consent form. I am now aware of the intended benefits, possible risks & complications, and available alternatives to the said procedure. I am aware that while majority of patients have an uneventful procedure and recovery, few cases may be associated with complications. I am also aware that results of any procedure can vary from patient to patient; and I declare that no guarantees have been made to me regarding success of the above procedure.

I also understand that these procedures may have to be performed more than once and has unique risks. I have had the opportunity to ask questions about the procedures listed and these questions have been answered to my satisfaction. I am also aware that I can withdraw my consent at any point of time at my own risk and consequence, by submitting the withdrawal in writing.

I am now also aware that during the course of these procedure the doctor will be assisted by medical and paramedical team, and that the doctor may seek consultation/assistance from relevant specialists if the need arises. I agree to observing, photography (still/video/televising) of the procedures (including relevant portions of my body) including my diagnosis/reports (pathology, radiology, etc), for academic/ medical/medico-legal purposes, provided my/my Patient's identity is not revealed by such acts. I also agree to my clinical details being shared for scientific publications provided my / my Patient's identity is not disclosed. I am also aware of the expected course after the procedure and the care to be provided and understand that sometimes extension of duration of hospitalization may be required and/or there may be requirement of extra medicines or treatments, thereby leading to increase in the treatment expenses, depending upon the body's response to the treatment/procedure.

I give my consent voluntarily to the members of the ICU team for ICU admission and for carrying out **Intubation and Mechanical Ventilation** that are integral to ICU admission on ☐ myself or ☐ my above named patient, being fully aware of the nature, potential risks and complications, intended benefits and possible alternatives.

I further declare that all fields (of this form) requiring insertion or completion were filled in my presence at the time of my signing this form. I, the above named Patient/named patient's representative, do further hereby declare that I am above 18 years of age as on the date of signing this form, mentally sound and am giving consent without any fear, threat or false misconception.

	Name	Signature	Date	Time
Patient				
Patient representative with relationship				
Reason for surrogate consent	Patient is unable to give consent because:			
Witness				
Interpreter (if necessary)				

only if Patient is a minor or unable to give consent

I, the undersigned doctor, have explained the nature, potential risks and complications, intended benefits, expected post-procedure course, and possible alternatives to the **Intubation and Mechanical Ventilation**, to the patient/patient representative. I am confident that he / she has understood the information fully as described in this document.

Consent obtained by^:

Dr. _____ Designation & Dept. _____ Signature: _____

^(being person performing the procedure or member of his /her team)